

Physician Endorsement Kit — The Caregiver's Cardinal Toolkit

Goal: 1 foreword (the prize) + 3–5 blurbs from working professionals. Priority order: a UNC/Duke geriatrician or palliative physician, an elder-law attorney, a hospital discharge planner or SHIP counselor, a dementia educator. Target list appended below (researched).

The rules that keep this clean

1. **Personal endorsements only.** A physician endorses with their own name, degree, and title — the book may never use a university's logo or imply the institution endorses it. Standard cover line: "*— Jane Smith, MD, Professor of Geriatric Medicine*" ✓ "*— Endorsed by UNC School of Medicine*" ✗
2. **No federal-title endorsements.** VA (and other federal) colleagues can't endorse a commercial product using their official titles — use them for warm introductions instead, which is worth more anyway.
3. **No payment for endorsements, ever.** A free finished copy is customary and expected.
4. Tell them exactly where the quote may appear (cover, Amazon listing, website) and let them approve the final wording of their own quote.

What goes in every ask (the packet)

- The one-pager below
- The finished manuscript PDF (or offer the 5 pages most relevant to them — busy clinicians often blurb from a targeted excerpt plus a skim)
- A specific deadline 3–4 weeks out
- The exact ask: "one to three sentences" (blurb) or "600–900 words" (foreword, offer editorial help)

The request letter (email template)

Subject: Would you consider endorsing a caregiver handbook? (2-minute ask)

Dear Dr. [Name],

I'm a Certified Dementia Practitioner who has spent my career in home health, hospice, and senior living here in North Carolina. After my father died, I wrote the book I kept wishing I could hand the families I sat with: *The Caregiver's Cardinal Toolkit* — a practical, color-coded workbook that gives families the exact questions, scripts, and worksheets for hospital discharges, care decisions, Medicare traps, and the last days of life.

Before publication, the manuscript went through independent clinical, Medicare/Medicaid, and legal reviews, and the medication content was reviewed by a Doctor of Pharmacy. [One sentence on why THIS doctor: their program, a paper, a talk, a shared contact.]

Would you consider providing a one-to-three-sentence endorsement? I'd send the full PDF today; if it would help, I can also point you to the five pages closest to your work. Any quote would appear with your name and title as you specify it (personal capacity — no institutional attribution), on the cover and the book's Amazon page, and you'd approve the final wording. I'd be grateful for an answer either way by [date].

Thank you for the work you do for older adults and their families.

Rob Brizzi, CDP Certified Dementia Practitioner · thecardinalspromise.com [phone] · [email]

(Foreword variant: swap the ask paragraph — "Would you consider writing a short foreword, 600–900 words? I'd be honored, I can share what readers most need framed, and my editor and I would handle any polishing you'd like.")

The one-pager (attach as PDF or paste below the letter)

THE CAREGIVER'S CARDINAL TOOLKIT — A Hero's Guide to Caring for an Aging Parent · Rob Brizzi, CDP · Cardinal Press · 208-page 8.5×11 color workbook · ISBN 978-1-963342-03-1 · Publication: [date]

One line: The map nobody hands a family on their worst day — 29 chapters color-coded by urgency, with word-for-word scripts, fillable worksheets, and five crisis pathways.

What reviewers verified: no false statements of current Medicare/Medicaid law; clinically sound fall, dementia, hospice, and last-days guidance; Jimmo, observation status, and the Part D cap handled correctly; medication content reviewed by Hope Brizzi, PharmD.

Who it's for: the 50+ million Americans caring for an aging family member — written for the 11pm hospital-hallway moment, not the armchair.

Why me: a career at the bedside in home health, hospice, and senior living; Certified Dementia Practitioner; founder of The Cardinal's Promise caregiver-education platform.

Sequencing that works

1. Warmest path first: ask your hospice/home-health colleagues "*who do you know at UNC or Duke geriatrics?*" — one forwarded email beats ten cold ones.
2. Ask blurbs from the professional tiers in parallel (attorney, discharge planner, SHIP counselor) — those land faster and make the physician ask feel safer ("others have already endorsed").
3. Ask ONE physician for the foreword only after a warm contact exists; everyone else gets the blurb ask.
4. When a quote lands: it goes on the back cover (top), the Amazon listing (Editorial Reviews), and the website — and the strongest one line goes on the FRONT cover under the subtitle.

Target #1 (author's warm contact): Dr. Dean Ferrera, Cardiologist — Indiana

Ask: blurb first (fastest yes); if the conversation goes well, he is also a candidate for the foreword. (Indiana-based — geography is irrelevant for an endorsement, and an out-of-state MD actually reinforces that this is a national book, not a local one.) Cover line on acceptance, in personal capacity: "*— Dean Ferrera, MD, Cardiologist*" (title/affiliation exactly as he specifies).

Why the book is in his lane (use in the letter): - Chapter 2's anticoagulated-fall guidance ("a head strike on a blood thinner means the emergency department the same day") — his patients - The medication-list system (Ch 25) and pharmacist notes — what he wishes every family brought to clinic - Palliative-care timing for advanced heart disease (Ch 11) — the referral conversation cardiology owns

Personalized letter — Dr. Ferrera

Subject: Would you consider endorsing my caregiver handbook, Dean?

Dear Dr. Ferrera,

[Personal opener — one sentence on how you know each other; Rob supplies.]

After my father died, I wrote the book I kept wishing I could hand the families I sat with through my years in home health, hospice, and senior living: *The Caregiver's Cardinal Toolkit* — a practical, color-coded workbook with the exact questions, scripts, and worksheets for hospital discharges, care decisions, Medicare traps, and the last days of life.

There's more cardiology in it than the title suggests: the fall chapter sends every anticoagulated head strike to the ED the same day, the medication-list system is built so a family can hand your team one accurate page, and the palliative chapter helps families hear "goals of care" the way you mean it. The manuscript passed independent clinical, Medicare, and legal reviews, and a PharmD reviewed the medication content.

Would you consider a one-to-three-sentence endorsement? I'll send the full PDF today — or just the ten pages closest to your clinic. The quote would run with your name and title exactly as you specify (personal capacity), on the cover and Amazon page, with your approval of final wording. Either way, thank you — and I'd be glad to send a copy for your waiting room regardless.

Rob Brizzi, CDP Certified Dementia Practitioner · thecardinalspromise.com [phone] · [email]

Target #2 (author's warm contact): Dr. Jason Huet, MD, Pulmonologist — Charleston

Ask: blurb (1–3 sentences). Cover line on acceptance, in personal capacity: "*— Jason Huet, MD, Pulmonologist*" (title/affiliation exactly as he specifies).

Why the book is in his lane (use in the letter):

- The home health chapter teaches families the homebound language his patients live — "leaving home causes significant weakness or shortness of breath" — so the evaluation actually gets ordered
- The hospice chapter shows families that hospice brings the oxygen, hospital bed, and supplies home — the referral conversation pulmonology owns for advanced COPD and lung disease
- The last-days pages walk families through breathing changes calmly: when to use the medicine or oxygen, and which signs mean urgent help

Personalized letter — Dr. Huet

Subject: Would you consider endorsing my caregiver handbook, Jason?

Dear Dr. Huet,

[Personal opener — one sentence on how you know each other; Rob supplies.]

After my father died, I wrote the book I kept wishing I could hand the families I sat with through my years in home health, hospice, and senior living: *The Caregiver's Cardinal Toolkit* — a practical, color-coded workbook with the exact questions, scripts, and worksheets for hospital discharges, care decisions, Medicare traps, and the last days of life.

There's more pulmonology in it than the title suggests: the home health chapter teaches families the homebound language your patients live — "leaving home causes significant weakness or shortness of breath" — so the evaluation actually gets ordered; the hospice chapter shows them that hospice brings the oxygen, hospital bed, and supplies home, which is the conversation that gets your advanced COPD patients referred in time; and the last-days pages walk a frightened family through breathing changes calmly — when to use the medicine or oxygen, and which signs mean urgent help. The manuscript passed independent clinical, Medicare, and legal reviews, and a PharmD reviewed the medication content.

Would you consider a one-to-three-sentence endorsement? I'll send the full PDF today — or just the pages closest to your practice. The quote would run with your name and title exactly as you specify (personal capacity), on the cover and Amazon page, with your approval of final wording. Either way, thank you — and I'd be glad to send a copy for your waiting room regardless.

Rob Brizzi, CDP Certified Dementia Practitioner · thecardinalspromise.com [phone] · [email]

Target list (researched)

Endorsement Target Research — "The Caregiver's Cardinal Toolkit"

A note on the "Carolina Alzheimer's Network"

Multiple targeted searches found no current, official UNC program by that exact name. The entities that occupy that space today are the **UNC Memory Disorders Program** (Division of Cognitive & Behavioral Neurology, med.unc.edu/neurology), the NIH-funded **Duke-UNC Alzheimer's Disease Research Center (ADRC)**

(dukeuncadrc.org), and the nonprofit **Dementia Alliance of North Carolina** (dementianc.org). Targets below are drawn from those.

Also a timing fact that matters: UNC's Division of Geriatric Medicine is in leadership transition. Founding chief **Jan Busby-Whitehead, MD** stepped down after 22+ years ([announcement](#)); **Cristin Colford, MD** is interim chief and was just named UNC SOM vice dean of education effective August 2026 ([UNC intranet](#)) — so the interim chief is the wrong ask, and the stable, senior names below are the right ones.

Primary target table (UNC), ranked by fit × likelihood

Rank	Name / Role / Program	Why them	Warm-path ideas	Source
1	Laura C. Hanson, MD, MPH — Professor & Vice-Chief for Research, Div. of Geriatric Medicine; Director/Medical Director, UNC Palliative Care & Hospice Program	The single best fit on paper: triple-boarded (IM, geriatrics, hospice & palliative medicine); her entire research program is palliative care for people with advanced dementia and their family caregivers , including the first multi-site palliative-care trial for late-stage AD/DRD (NCT04948866); INPCS Outstanding Scientific Contribution Award for dementia palliative care (UNC DOM). A hospice + dementia + caregiver workbook is squarely her mission.	Rob's hospice career: any UNC Hospice / UNC Health palliative colleagues, hospice medical directors, or IDG physicians he's worked with in the Triangle almost certainly know her. The Palliative Care & Hospice Program has a public directory/program contact (med.unc.edu/pcare). Lead with the hospice chapter and caregiver worksheets.	UNC Geriatrics profile
2	Andrea Bozoki, MD — Professor & Chief, Division of Memory and Cognitive Disorders, UNC Neurology; Outreach & Engagement Core Leader, Duke-UNC ADRC	UNC's most visible dementia specialist, with the strongest <i>public-education</i> signals of anyone on this list: she gives caregiver-facing community talks hosted by the Duke Dementia Family Support Program (2024 flyer , 2021 flyer), headlines the UNC Health Foundation "Aging Brain Health" donor luncheon (unchealthfoundation.org), and does coalition interviews (NC SIC). Her ADRC core exists to do community/caregiver education — endorsing a vetted caregiver toolkit is on-mission.	Attend one of her public talks (Duke Family Support Program calendar) and ask in person; or route through the ADRC Outreach Core's community events (dukeuncadrc.org); or via UNC Memory Disorders Clinic program contact (UNC Neurology team page).	UNC Neurology
3	Karen Halpert, MD — UNC Family Medicine & Geriatric Medicine (Chapel Hill/Durham clinics)	Already does exactly this genre of consumer communication: she is the featured expert in UNC Health Talk's " 5 Tips for Tough Conversations with Aging Parents " — advance directives, goals-of-care talks, the book's exact territory. Practicing community geriatrician = credible to lay readers; media-willing = high yes-rate. Less national "name" than Hanson/Bozoki but arguably the most likely yes.	Cold-warm ask citing her Health Talk piece ("your five tips are the same conversation my Chapter X scripts"); her Durham clinic overlaps Rob's geography and home-health referral networks. Faculty directory: UNC Family Medicine .	UNC Fam Med
4	Meredith Gilliam, MD, MPH — Vice Chief of Clinical Operations, UNC Geriatric	Runs UNC's flagship geriatrics clinic; performs cognitive/dementia assessments; has published on discussing deprescribing with caregivers of people with dementia — a direct hook to the	The Eastowne clinic sees the families Rob's book serves — offer copies as a clinic caregiver resource alongside the blurb ask;	UNC Geriatrics profile

Rank	Name / Role / Program	Why them	Warm-path ideas	Source
	Medicine; Medical Director, UNC Geriatrics Eastowne Clinic	PharmD-reviewed medication chapters. Featured in a Department of Medicine " Physician Spotlight ". Rising-leader profile: motivated, reachable.	home-health liaisons who market to UNC Geriatrics Eastowne will know her.	
5	Jan Busby-Whitehead, MD — Founder & former Chief (22 yrs), UNC Geriatric Medicine; Director, UNC Center for Aging and Health ; PI, Carolina Geriatric Workforce Enhancement Program (CGWEP)	The legacy name in NC geriatrics; retained the Center for Aging and Health directorship after stepping down as chief (UNC DOM). CGWEP is a HRSA education program whose mission includes training family caregivers (med.unc.edu/aging/cgwep) — a caregiver workbook fits its purpose. Post-chief, she may have more bandwidth; a founder's blurb carries statewide weight.	Route through the Center for Aging and Health / CGWEP program office (staff page , co-director Ellen Roberts, PhD, MPH); CGWEP runs community trainings Rob could offer to support.	Center for Aging and Health

Also flagged: **Alyssa Tilly, MD** (UNC Hospice & Palliative Medicine Fellowship Program Director, [program leadership](#)) is a reasonable backup palliative ask if Hanson declines.

Duke alternates (the adjacent bench)

Name / Role	Why them	Notes / warm paths	Source
David Casarett, MD — Chief, Section of Palliative Care, Duke; directs Duke Center for Palliative Care	The one physician in the Triangle who is <i>himself a trade-book author</i> ("Shocked," "Stoned," "Undiscovered Country: A Doctor's Travel Guide to the End of Life"; TED speaker) — he understands blurb requests, hospice content, and lay audiences, and has no fear of public-facing work. Arguably the highest-likelihood physician "yes" in the region.	Ask as author-to-author: reference "Undiscovered Country" (end-of-life for lay readers). Duke Palliative Care leadership page: medicine.duke.edu .	Simon & Schuster author page , Duke restructuring news
Heather Whitson, MD, MHS — Director, Duke Center for the Study of Aging and Human Development (Duke Aging Center); Co-Director, Duke-UNC ADRC	Leads the nation's longest continuously funded aging center; geriatrician; does public media (" Caring for the Aging Brain ," Duke Today). ADRC co-director title spans both universities.	Duke Aging Center people page: agingcenter.duke.edu/people . Busy; make the ask tiny and deadline generous.	Duke profile
Cathleen Colón-Emeric, MD — Chief, Duke Division of Geriatrics and Palliative Care	Runs the combined division (restructured Sept 2024) — the senior geriatrics title at Duke.	Division leadership page	Duke news
Duke Dementia Family Support Program — Lisa P. Gwyther, MSW (founder/director, Assoc. Prof. Psychiatry); Bobbi	The most caregiver-facing program in NC since 1980; NC Division of Aging-funded since 1984; Gwyther co-authored <i>The Alzheimer's Action Plan</i> — a caregiver-book author who reviews exactly this	Program hosts free public events (where Bozoki speaks — two birds); it also houses Project C.A.R.E.'s Central NC office (919-660-7510). Staff page: dukefamilysupport.org/about-us/staff .	About, Duke Today profile

Name / Role	Why them	Notes / warm paths	Source
Matchar, MSW, MHA (senior advisor)	material. Not physicians (MSW), so this is a <i>credibility</i> blurb, not the physician blurb — but their newsletter/events are also a distribution channel.		

NC statewide non-university validators (one ask each)

Org / Person	What they are	Angle & caveat	Source
Project C.A.R.E. — NCDHHS Division of Aging and Adult Services	NC's state-funded dementia-caregiver consultation program; regional offices in Asheville, Hickory, Wilmington, Washington, and Durham (housed at Duke Family Support Program, 919-660-7510)	Ask the Central NC office consultant for a practitioner's quote; a state agency won't "endorse," but a named care consultant can. Also the natural bulk-buyer/recommender of the workbook.	NCDHHS Project CARE
Alzheimer's Association — Eastern NC Chapter: Christine John-Fuller, Executive Director; Katherine Lambert, Regional Leader NC/SC/GA (CEO, Western Carolina Chapter)	Covers the 51 eastern counties incl. Durham/Triangle	Caveat: the national Association generally does not endorse third-party products/books — expect a personal-capacity quote at best, or ask instead to present at chapter caregiver-education events (worth more than a blurb).	alz.org/nc/about_ED_announcement
AARP North Carolina — Michael Olender, State Director (919-508-0265, molender@aarp.org per AARP's own contact page)	1.1M members statewide	AARP brand endorsement is essentially unobtainable (strict licensing); the realistic win is a personal quote, an AARP NC caregiving-event slot, or local-article coverage.	AARP NC contact page
Association for Home & Hospice Care of NC — Tim Rogers, President & CEO (since 2001)	The state trade association for Rob's own industry	Highest warm-path density: Rob's current/former employers are almost certainly members; a Rogers quote validates the home-health/hospice chapters and opens AHHC conference exhibiting/speaking.	AHHC staff page
NC SHIIP — Melinda Munden, Director, Seniors' Health Insurance Information Program, NC Dept. of Insurance (Melinda.Munden@ncdoi.gov ; 855-408-1212)	NC's Medicare counseling program; Munden won a national SHIP award in 2023	A state regulator won't blurb, but SHIIP is the perfect <i>accuracy validator</i> for the Medicare-traps chapters — ask for a technical read, then say "reviewed with input from NC SHIIP counselors" (with their permission). Also: 400+ county SHIIP sites are a distribution channel.	NCDOI press release, SHIIP contact

How these asks actually succeed

What to send (one email, complete): finished, paginated PDF (not a draft) + a one-page sheet: synopsis, TOC, trim size/page count/price/pub date, author bio leading with the CDP credential and 20+ years in home health/hospice, and — critically for physicians — the review pedigree ("medication content reviewed by [PharmD name]; manuscript twice expert-reviewed"). Offer 2–3 *sample blurbs* they can edit rather than write from scratch, state exactly where the blurb will appear (back cover, Amazon page, press materials — nothing else), and give a specific deadline 4–8 weeks out with one polite reminder two weeks before. This matches standard publishing practice ([Jane Friedman](#), [Clear Sight Books](#)).

Why academic physicians decline — and how to pre-empt it: the #1 killer is *implied institutional endorsement*. University COI policies (e.g., [Univ. of Maryland](#), [OU HSC](#)) bar faculty from presenting their title in ways suggesting the university endorses an outside product. Pre-empt it in the ask: "Your affiliation would appear for identification only, e.g., 'Jane Doe, MD, Professor of Medicine' — no UNC logo, no implication of UNC endorsement, and you approve the exact credit line." The #2 killer is commercial conflict — reassure them the book sells no services, recommends no paid products, and that they receive nothing of value (a free copy is fine). The #3 is accuracy risk on medical content — the PharmD review and offer to accept their line-edits directly answers it.

Foreword vs. blurb: a blurb (2–3 sentences, ~30–60 min of their time) is a low-stakes yes; a foreword is a co-branding decision — it puts their name *inside* the book, usually requires reading the whole manuscript, sometimes involves an honorarium, and triggers much more COI caution. Ask for the blurb; if someone (Hanson or Casarett) responds warmly, *then* float the foreword as an upgrade they can decline gracefully.

Sequencing that exploits this list: (1) get the easy statewide/practitioner quotes first (AHHHC's Rogers, a Project C.A.R.E. consultant, Duke Family Support) — they become social proof; (2) then ask Halpert and Gilliam with those quotes attached; (3) then the marquee asks — Hanson, Bozoki, Casarett — with a package that already looks endorsed. In-person beats email: Bozoki and the Duke Family Support Program hold free public caregiver events in Durham, minutes from Rob — attending one and asking afterward, book in hand, is the single highest-yield move on this list.

Verification caveats: all names/titles are as published on the cited official pages as of July 2026; UNC's med-school pages block automated fetching, so several UNC titles were confirmed via search excerpts of official pages rather than full page reads — Rob should confirm current titles on each linked page immediately before sending (especially given the geriatrics chief transition, where the permanent-chief search was still open at last report).